tere

Tere Health

AGED RESIDENTIAL CARE · AOTEAROA NEW ZEALAND

A doctor for your residents, **without the transfer.**

- HR · RR · SpO_2 · BP over video
- Doctors 8am–8pm, seven days
- Queue open 24/7
- Script, imaging, ACC and GP letter in the consult

Acute and after-hours medical cover for rest home, hospital-level and dementia residents, delivered by video with your nurse in the room.

Most transfers aren't about the ambulance. They're about **not having a doctor.**

When a resident goes off at seven in the evening, the decision isn't really clinical. It's that nobody available can lay eyes on them. **That is the gap we fill**, and it matters most in a secure dementia unit, where a night in a strange room is its own harm.

TODAY · RESIDENT FALLS AT 7PM. GRAZE, AND A SORE HIP.

1. RN assesses. GP rooms are shut. The after-hours options are an on-call service or 111.
2. Ambulance to the nearest hospital. An RN or a family member goes with them.
3. Back in the small hours, disoriented. Everyone is tired and nothing was broken.

A night in ED for something that needed a doctor's eyes.

WITH TERE · SAME RESIDENT, SAME FALL

1. RN opens your page on the tablet and joins the queue. Carries on with the shift.
2. Doctor assesses by video with your RN in the room. Heart rate, breathing, oxygen and blood pressure captured through the camera.
3. X-ray referral if it's warranted, ACC lodged, notes to the resident's own GP that evening.

The resident stays in their own bed unless they genuinely need to leave it.

WHO THIS PROTECTS

- **The resident.** No ambulance, no strange ward, and for a dementia resident, no night of delirium risk that can take weeks to come back from.
- **Your staff.** No RN off the floor escorting a transfer, which is the part that actually costs you on a night shift.
- **The family.** A doctor assessed them the same evening, and the decision not to transfer was made by a doctor, not by a roster gap.
- **The facility.** A documented clinical assessment and a decision trail, which is exactly what a Ngā Paerewa audit wants to see.

WHAT THIS IS NOT

We do not replace your contracted GP. Routine reviews, medication charts, long-term condition management and the three-monthly cycle stay exactly where they are. We cover the gap: after hours, weekends, and the acute problem that cannot wait for the next scheduled visit.

Your nurse opens one page. **That is the whole process.**

No app to install, no account for each staff member to remember, no password reset at two in the morning. It runs on the tablet or laptop you already have on the floor.

BEFORE GO-LIVE · ONCE

1. **You send us a CSV of your residents.** Name, date of birth, NHI, GP practice, and anything standing we should know. We give you the template, and we build every record before you go live, so nobody is typing details into a form at 7pm.
2. **We build your page.** A page at terehealth.co.nz carrying your name, reachable only by your staff.
3. **You update it monthly.** Send a fresh CSV as residents come and go, or email us the changes and we will do it.

ON THE NIGHT · EVERY TIME

4. **Open your page and pick the resident** from your own roster. Their details are already there.
5. **Join the queue and carry on.** We ring the number you nominate when a doctor is ready, so nobody stands holding a tablet.
6. **Your RN joins the consult.** The doctor assesses with your nurse in the room, which is better medicine than a resident on their own with a screen.

WHAT COMES OUT OF THE CONSULT

- **Prescription** sent to your usual pharmacy.
- **X-ray or ultrasound referral** written during the consult, not the next working day.
- **ACC claim lodged** for you where the problem is an injury.
- **A letter to the resident's own GP** the same evening, so continuity is kept and your GP is not surprised.
- **Clinical notes** you can file against the resident's record for your own documentation.

NO CARD AT THE BEDSIDE

Your staff never take a payment, never enter a card, and never chase a family for one. **Everything goes on a single monthly invoice**, itemised by resident and date. This is usually the part rest homes tell us matters most.

You pay for what you use. **Nothing else.**

No subscription, no per-bed fee, no minimum volume and no joining cost. If you have a quiet month, you pay almost nothing.

$65

Standard consultation

Any medical problem inside our scope. One flat fee whatever the hour, including weekends and public holidays.

$25

ACC consultation

Where the problem is an injury and the claim is accepted. Falls, knocks, skin tears. We lodge the claim as part of the consult.

One invoice, month end

Itemised by resident

Payable on your normal terms. No card at the bedside, no staff member out of pocket, no reimbursement paperwork.

AN ILLUSTRATIVE MONTH

ITEM	VOLUME	RATE	TOTAL
Standard consultations	8	$65	$520
ACC consultations (falls, skin tears)	4	$25	$100
Resident records loaded and maintained	—	$0	$0
Your dedicated page and monthly reporting	—	$0	$0
Invoiced at month end	**12**		**$620**

Volumes shown are an example for sizing, not a forecast. We would agree a realistic baseline with you after the first month.

WHAT YOU ARE COMPARING IT AGAINST

- The cost of an RN leaving the floor to escort a transfer, and backfilling that shift.
- After-hours medical fees for a call-out, where one is even available in your area.
- The clinical cost of a dementia resident spending a night in an unfamiliar environment.

We have deliberately not put a dollar figure on these. **You know your own numbers better than we do**, and we would rather you ran them than took ours.

NO LOCK-IN

No term, no notice period, no exit fee. If it is not earning its place after three months, stop sending residents to us and the invoices stop.

You will know exactly what we did, **and for whom.**

A report lands with your manager every month, built for your clinical governance meeting and for your auditor. The invoice reconciles to it line for line.

CONSULTS PER MONTH · ILLUSTRATIVE



Winter peaks are what we would expect in a rest home. Seeing the shape of your own year is half the value of the report.

PER-RESIDENT TALLY · ILLUSTRATIVE

RESIDENT	CONSULTS	ACC	LAST SEEN
Resident A · dementia unit	4	2	12 Jun
Resident B · rest home	3	0	28 Jun
Resident C · rest home	2	1	19 Jun
Resident D · day care	1	0	04 Jun
June total	**19**	**6**	

WHAT ELSE IS IN IT

- **Reason for each consult**, grouped, so you can see whether falls or infections are driving your volume.
- **How many ended in a transfer anyway**, which is the honest measure of whether this is working.
- **Invoice that reconciles** to the tally, line for line.
- **Ask us any time** and we will send it, not just at month end, if your manager wants it mid-month for a meeting.

Four things no other NZ telehealth service **offers together.**

VITALS OVER VIDEO

MEDSAFE WAND CLASS IIa · SPOT-CHECK · HR · RR · SpO_2 · BP

The camera reads a real heart rate, respiratory rate, oxygen saturation and blood pressure. For a resident who will not tolerate a cuff, or a dementia resident who finds being handled distressing, this is often the difference between a usable set of observations and none. **No other NZ telehealth service captures BP over video.**

LIVE LANGUAGE SUBTITLES

15 LANGUAGES · REAL-TIME · CONSENT-GATED

Speech is transcribed into the chosen language on screen during the consult, both ways. Te Reo Māori, Tagalog, Samoan, Tongan, Hindi, Mandarin, Russian and more. **In aged care this matters as much for your staff as your residents**, since the person describing the fall to the doctor is often an internationally qualified carer on a night shift.

DOCTORS 8AM TO 8PM, SEVEN DAYS

QUEUE OPEN 24/7 · INCLUDING WEEKENDS AND PUBLIC HOLIDAYS

A rostered doctor every day of the year, twelve hours a day, including the long weekends when your GP rooms are shut and your staffing is thinnest. The queue itself never closes, so your nurse can put a resident in it at any hour. **Anything that cannot wait is diverted to 111 at triage**, before anyone joins a queue.

EVERYTHING SORTED INSIDE THE CONSULT

NO NEXT-WORKING-DAY LOOSE ENDS

Prescription to your pharmacy, imaging referral written while the doctor is still on the call, ACC lodged, and a letter to the resident's own GP the same evening. **Your nurse does not inherit a to-do list** at the end of the consult, which is the usual reason telehealth creates work for a rest home instead of removing it.

Our vitals, measured.

580 reference scans across 455 subjects, both live-capture and research stress-test. The algorithm that goes to every patient, measured against pulse oximeter.

HEART RATE · MAE

5.1 bpm

181 live-capture scans vs reference pulse oximeter. Median error 3 bpm. Blood pressure accuracy on the next slide.

HR within **5 bpm** of reference	**71%**
HR within **10 bpm** of reference	**90%**
Unique subjects (full validation)	**455**
Capture window	**10 seconds, 4 passes**

RESPIRATORY RATE

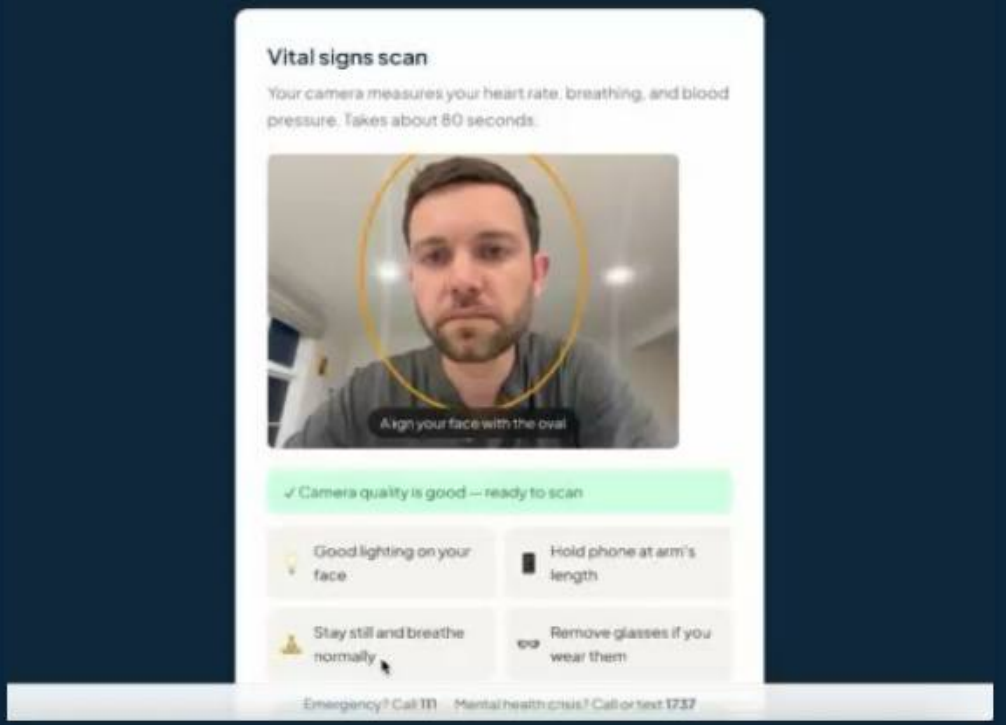
Three-source fusion (AM + FM + baseline wander). **78% reduction** in implausible readings on live scans since Sept 2026.

SPO₂

Per-person calibration against a reference oximeter, persisted and portable. Low-confidence readings are hidden rather than guessed.

MedSafe WAND Class IIa. Adult 18+, spot-check, non-diagnostic. Vitals are an indicative input to a clinician, not a replacement for medical-grade devices. Validation set is open and growing.

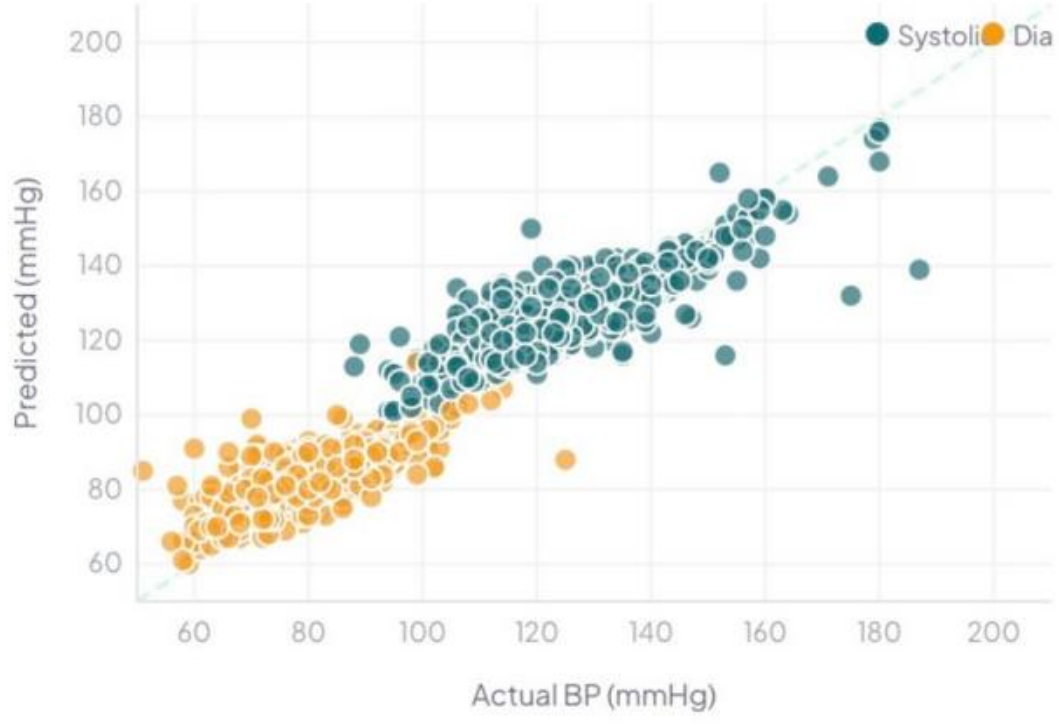
LIVE DEMO · 1 MIN 45 SEC



What the patient sees. Phone camera reads heart rate, respiratory rate and blood pressure in 10 seconds. Full video plays in the live deck; this is the end-of-capture screen.

Accurate in the clinical sweet spot.

Predicted vs actual blood pressure on 116 unseen validation scans. Dashed line = perfect prediction. Strongest in the normotensive range; wider error at the extremes, where a cuff follow-up is already indicated.



v3 model · POS + multi-pass windowing · face-only · 100 trees

VALIDATION MAE · 116 UNSEEN SCANS

±11.4 mmHg — SYSTOLIC

±9.7 mmHg — DIASTOLIC

Mean absolute error on scans the model never saw during training.

FOR COMPARISON · AAMI SP10 CUFF STANDARD

Validated BP cuffs are permitted up to **±5 mmHg mean error** and **±8 mmHg SD** against a reference. We're **±11.4 mmHg** from a phone camera — no cuff, no touch.

MedSafe WAND Class IIa. Adult 18+, spot-check, non-diagnostic. Any reading outside the normotensive band prompts an in-person cuff check.

A registered NZ telehealth service. Built to sit alongside the facility's contracted GP as the after-hours and acute pathway, not in place of them.

Tere Health Ltd operates a video-first, MCNZ-supervised general and emergency-medicine practice, delivered entirely from Aotearoa, with the clinical and regulatory footprint of a bricks-and-mortar clinic.

→ **All clinicians hold current NZ APCs** and practise under MCNZ Telehealth Statement (2020).

→ **Emergency Medicine & General Practice**, designed for the presentations that stretch a rest home after hours.

→ **Registered as a healthcare provider** with HPI-O identifiers on both facility and clinicians.

→ **Owned and operated in NZ**, no offshore call centre, no US doctors reading transcripts.

REGULATORY POSTURE AT A GLANCE

Medicines Act (Rx)	Yes, Section 25 prescribing
ACC contracted	Yes, provider ID active
MedSafe vitals class	WAND Class IIa (HR / RR / SpO_2 / BP)
Privacy Act 2020	Full breach runbook + IPP12 notices
HDC Code	All 10 Rights operationalised
HISO 10029	Conformance evidence pack maintained

What we treat, and what we always divert.

IN SCOPE, TYPICAL DAILY LOAD

- UTIs
- Acute respiratory
- Skin infections
- MSK / ACC injuries
- Minor mental health
- Repeat prescriptions
- Sick certificates
- Contraception
- Post-discharge follow-up
- Rural workplace injury triage

Prescribing today: encrypted PDF sent directly to the patient's chosen pharmacy, drawn from the community dispensary contact directory Tere maintains for every registered NZ pharmacy. NZePS integration pending HNZ access approval.

OUT OF SCOPE, HARD DIVERT TO 111 / ED

- → Chest pain, cardiac symptoms, syncope
- → Stroke symptoms (FAST positive)
- → Anaphylaxis or airway compromise
- → Acute abdomen, GI bleed, obstructed labour
- → Suicidal intent, acute psychosis, DV / safety threat
- → Paediatric < 2 years, high-acuity paeds
- → Anything the AI-triage or clinician flags as **needs eyes and hands**

Divert list is system-enforced at triage, patient cannot progress past it.

Same safety net as a bricks-and-mortar clinic, often tighter, because it's coded.

→ **Imaging & specialist referral follow-up**, nightly reconciliation chases every outbound referral until the result lands back and is reviewed.

→ **Safety-netting at every close**, provider must specify red-flags-to-return-for before finalising notes.

→ **72-hour re-presentation surveillance**, automated flag if the same complaint returns to us or the patient's GP.

→ **Break-glass audit**, every chart open outside an active queue requires written justification and hits an admin surface.

→ **Prescribing surveillance**, cross-provider max-quantity and early-refill guards for controlled and abusable drugs.

→ **Clinical Governance Meeting framework**, cadence tracker + structured minutes, ready for weekly meetings from launch.

→ **Peer review + countersign**, RMOs and NPs have named supervisors; sign-off gated in the software, not on paper.

→ **HDC Right 10 workflow**, 20-day complaint timeline tracked in-app; Advocacy Service auto-cited in every response.

→ **Compliance training**, annual PHI + cultural safety attestations with nightly expiry monitoring; APC and medical indemnity gate first-time chart access.

→ **Business-continuity fallback**, static "platform down" phone banner + failover contact tree.

Your patients' data doesn't leave New Zealand. And it's been independently tested.

INDEPENDENT PENETRATION TEST

No critical vulnerabilities identified.

Blacklock Security Limited

Tested 29 September 2026

Standards: OWASP v4.2 · PTES · OSSTMM

Full certificate + report available under NDA.

SOVEREIGN HOSTING

Supabase (Auckland) + AWS Sydney under BAA. No US processing of PHI.

IMMUTABLE BACKUP

S3 Object Lock, 7-year Compliance retention. Ransomware-proof.

MFA MANDATORY

Every clinician account. Hard block on chart access without TOTP enrolment.

COLUMN-LEVEL ENCRYPTION

pgcrypto on highest-sensitivity PHI columns. Encrypted at rest and in transit.

FULL AUDIT TRAIL

Every PHI view logged with reason. Break-glass access requires written justification.

Certificates, letters, and audit artefacts, available under NDA.

MEDSAFE WAND

Class IIa vitals certificate covering HR, RR, SpO_2 and BP (adult spot-check, non-diagnostic). Reference 260729-WAND-786DQ9.

INDEPENDENT PENETRATION TEST

Blacklock Security, 29 Sep 2026. OWASP v4.2 + PTES + OSSTMM. No critical vulnerabilities identified.

HDEC / NEAC

HDEC out-of-scope letter held for vitals validation study; NEAC 2019 National Ethical Standards applied to all internal research.

DATA HOSTING

Sovereign NZ + Sydney, Supabase (Auckland), AWS Sydney (Bedrock, SES, SNS, S3) under BAA. No US processing of PHI.

IMMUTABLE BACKUP

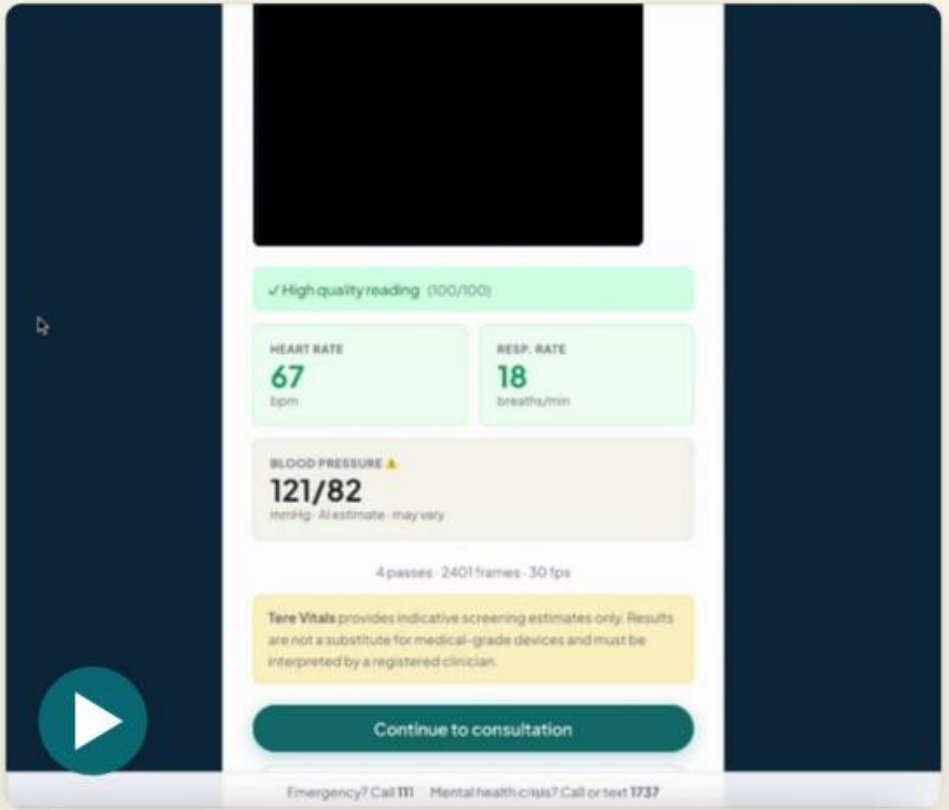
S3 Object Lock, 7-year Compliance retention in a segregated AWS account. Nightly. Ransomware-proof. Restore drill tested.

ISO 27001 PATHWAY

Phase 1 complete, phase 2 controls in progress. Full compliance evidence pack maintained for regulator audit or operator due diligence.

What the patient and clinician **actually see.**

A sixty-second walkthrough of a real consult, end to end: language choice, the vitals scan, the doctor, and what gets sent afterwards.



SCAN WITH YOUR PHONE CAMERA

Watch the walkthrough

terehealth.co.nz/watch

- **No sign-up** and nothing to install to watch it.
- **Sixty seconds**, and it is the quickest way to show your nurses what the consult looks like.
- Happy to **walk through it live** with your clinical lead instead.

We could have you live **inside a fortnight.**

Nothing here needs procurement, an IT project or a change to your existing GP arrangement.

1. **A thirty-minute call.** We walk your manager and clinical lead through an actual consult, end to end, and you tell us where it would break on your floor.
2. **You send the resident CSV.** We provide the template. Name, date of birth, NHI, GP practice, standing notes.
3. **We build your page.** Two to three working days.
4. **Twenty minutes with your RNs** on the floor. Not a training course, and not a module anyone has to sit.
5. **Go live, then review at thirty days** against your own numbers: consults, transfers avoided, and what it cost you.

WHAT WE WOULD NEED FROM YOU

- A named clinical contact and the number we ring when a doctor is ready.
- Your resident roster as a CSV, and someone who owns keeping it current.
- Your GP practice's details, so our letters land in the right inbox.
- A decision on whether the dementia unit is in from day one or added later.

CONTACT

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